WEEK
8

Week 8 Study Guide

15 Collaterals · 12 Divergent Channels · 12 Muscle Regions · 12 Cutaneous Regions

No new organ acupoints this week -- structural/conceptual layer built on top of the 12 Primary Meridians

This Week Covers:

- The Three Circuits framework (Outer/Anterior, Inner/Posterior, Middle) organizing the whole lecture
- The 15 Collaterals (Luo-Connecting points) -- all 12 paired points + the 3 that bring the total to 15
- The 12 Divergent Channels -- the Li-He-Chu-merge (beginning/organs/exiting/merging) framework, all 6 confluence pairs
- The 12 Muscle (Sinew) Regions -- pathway, binding points, and the 4 structural pattern rules
- The 12 Cutaneous Regions -- theory, function, and the 3-Yang / 3-Yin lecture diagrams
- Week 7 confluent-points review + Dr. Zhang's own exam-focus summary (verbatim, slide 84)
- Flagged: homework/quiz status discrepancy (slide text vs. verbal statement) -- see notes

Dr. Zhang stated verbally that Week 8 has NO new homework, and Week 9 (review week) has NO new quiz -- Week 9 is pure review before the final exam. Slide 85's "Homework2 Due Week 8 / Quiz 2" text conflicts with this and is flagged as likely stale template text, not silently corrected.

Written-syllabus Week 8 reading entry not available in project files (flagged, not verified). Slide 85 lists MOA p.16-17, 26-28 + intro to the 12 Primary Meridians and CAM p.88-114 under "For Next Week," but this appears to be recycled template text (see discrepancy note in wk8_content.py) -- confirm with Dr. Zhang before treating as assigned.

Slides: Dr. Vivian Zhang, Lecture8vivian1119.pdf (85 slides; 1-61 delivered live, 62-85 self-study)

Dr. Vivian Zhang, Ph.D. | Jon Centeno | D.AcHM Candidate | VUIM

The Three Circuits -- Dr. Zhang's Organizing Structure

Every circuit follows the same four-leg rule: the Yin meridian of the Hand runs chest->hand; its paired Yang meridian of the Hand runs hand->head; the next Yang meridian, of the Foot, runs head->foot; and the paired Yin meridian of the Foot runs foot->abdomen/chest, closing the loop back to the next Yin-Hand meridian. Outer Circuit = LU->LI->ST->SP. Inner Circuit = HT->SI->BL->KI. Middle Circuit = PC->SJ->GB->LR.

Outer Circuit (Anterior) (Yangming / Taiyin)

Pairs: LU / LI | ST / SP

Chest->Hand->Head->Foot->Chest. Anterior aspect of the body.

Inner Circuit (Posterior) (Taiyang / Shaoyin)

Pairs: HT / SI | BL / KI

Chest->Hand->Head->Foot->Chest, posterior aspect. BL is the largest meridian (67 pts, 6 branches).

Middle Circuit (Lateral) (Shaoyang / Jueyin)

Pairs: PC / SJ | GB / LR

Lateral aspect. GB/LR muscle regions + cutaneous regions were slide-deck content not reached live (self-study).

This Week's Four New Layers (all built on TOP of the 12 primary meridians)

- 15 Collaterals (Luo Mai): *ON the surface -- links each paired meridian via one Luo-Connecting point.*
- 12 Divergent Channels (Jing Bie): *run DEEP inside -- no points, no pertaining organ; reinforce the organ relationship.*
- 12 Muscle/Sinew Regions (Jing Jin): *cover a WIDE band along each meridian's course -- muscle/joint function only, no organs.*
- 12 Cutaneous Regions (Pi Bu): *the OUTERMOST layer -- where meridian qi is reflected on the skin surface.*

Depth Progression -- Surface to Core

Su Wen's disease-transmission order (used again on the Cutaneous Regions page) doubles as a map of how deep each of this week's four systems sits, from the skin inward:

1. Cutaneous Regions		2. Collaterals		3. Primary Meridians		4. Divergent Channels
outermost -- qi reflected on the skin surface (Pi Bu)	->	*surface -- links paired meridians via one point each (Luo Mai)*	->	*the main channels themselves, with their full point sets*	->	*deepest -- no points, reinforce organ links (Jing Bie)*

Note: Muscle/Sinew Regions Sit Outside This Ladder

Muscle Regions (Jing Jin) don't fit neatly into the surface-to-core depth ladder above -- they cover a WIDE BAND along each meridian's course (muscle/joint tissue specifically), rather than sitting at one consistent depth. Think of them as a separate, parallel layer keyed to function (movement) rather than to depth.

The 15 Collaterals (Luo Mai) -- Definition & Logic

The 12 Primary Meridians and the Conception and Governor Vessels each give off a collateral branch, totaling fifteen when combined with the Major (Great) Collateral of the Spleen -- collectively the “15 Collaterals” (Luo Mai). Each is named after the acupoint from which it originates -- its Luo-Connecting point.

Why 15, Not 12?

Why 15, not 12? The 12 paired meridians (6 yin-yang pairs) each need a Luo-Connecting point to link them -- that's 12. But the 12 primary meridians don't have their own acupoints EVERYWHERE harmony is needed: two extra vessels (Du Mai/GV and Ren Mai/CV) have their own dedicated points, so they each need their own collateral too -- that's 14. And there is one region no collateral yet covers: the LATERAL side of the chest (CV covers front, GV covers back, but nothing covers the side). The Spleen's Great Collateral (Dabao, SP 21) fills that gap -- bringing the total to 15.

Function -- What a Luo-Connecting Point Does

Function: the Luo-Connecting point's job is to link (□, luo = “net/connect”) each meridian to its interior-exteriorly paired partner, strengthening their shared function and treating disorders that involve both. Distinguish from a Yuan-Source point (treats disorders of its OWN meridian) and from a Confluent point (opens an Extraordinary Vessel). One acupoint can carry more than one of these special-point identities at once (e.g. LU 7 is both Luo point AND CV's Confluent point) -- when a point carries multiple special functions, its clinical reach is broader/stronger.

The 12 Paired-Meridian Luo-Connecting Points

MERIDIAN	LUO POINT	CONNECTS TO (PAIR)
Lung (LU)	LU 7 Lieque	*Large Intestine*
Large Intestine (LI)	LI 6 Pianli	*Lung*
Stomach (ST)	ST 40 Fenglong	*Spleen*
Spleen (SP)	SP 4 Gongsun	*Stomach*
Heart (HT)	HT 5 Tongli	*Small Intestine*
Small Intestine (SI)	SI 7 Zhizheng	*Heart*
Bladder (BL)	BL 58 Feiyang	*Kidney*
Kidney (KI)	KI 4 Dazhong	*Bladder*
Pericardium (PC)	PC 6 Neiguan	*Sanjiao (Triple Energizer)*
Sanjiao (SJ)	SJ 5 Waiguan	*Pericardium*
Gallbladder (GB)	GB 37 Guangming	*Liver [self-study]*
Liver (LR)	LR 5 Ligou	*Gallbladder [self-study]*

[self-study] = GB/LR collaterals were slide-deck content Dr. Zhang did not reach live this lecture.

Rule of Thumb (from lecture)

Luo-Connecting points of HAND meridians cluster around the wrist. Luo-Connecting points of FOOT meridians cluster around the ankle (malleolus). Use this to sanity-check any point location you're unsure of.

Special-Point Identity Overlaps (from Week 7 -- these are the same points!)

POINT	AS A LUO POINT	ALSO SERVES AS
LU 7 Lieque	Luo-Connecting point of Lung	*Confluent (opening) point of Ren Mai (CV)*
SP 4 Gongsun	Luo-Connecting point of Spleen	*Confluent (opening) point of Chong Mai*
PC 6 Neiguan	Luo-Connecting point of Pericardium	*Confluent (opening) point of Yin Wei Mai*
SJ 5 Waiguan	Luo-Connecting point of Sanjiao	*Confluent (opening) point of Yang Wei Mai*

This is exactly the “one point, multiple jobs” principle Dr. Zhang flagged in Week 7 -- a point's clinical reach grows with each special-point identity it carries.

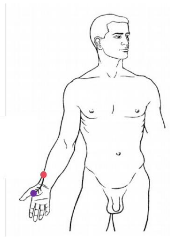
Collateral Pathway Diagrams -- Outer & Inner Circuit

The Collateral of Outer Circuit(1)

- **The Collateral of the Lung Meridian of Hand-Taiyin**

It arises from Lieque (LU 7) and runs to the Large Intestine Meridian of Hand Yangming.

Another branch follows the Lung Meridian of Hand Taiyin into the palm of the hand and spreads through the thenar eminence.



Lung

19

Lung (LU)

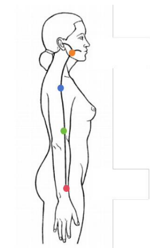
LU 7 Lieque

The Collateral of Outer Circuit(2)

- **The Collateral of the Large Intestine Meridian of Hand-Yangming**

It starts from Pianli (LI 6) and joins the Lung Meridian of Hand-Taiyin three cun above the wrist.

Another branch runs along the arm to Jianyu (LI 15), crosses the jaw and extends to the teeth. Still another branch derives at the jaw and enters the ear to join the Thoroughfare Vessel.



Large Intestine

20

Large Intestine (LI)

LI 6 Pianli

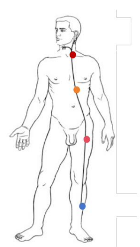
The Collateral of Outer Circuit(3)

- **The Collateral of the Stomach Meridian of Foot-Yangming**

It starts from Fenglong (ST 40), eight cun above the external malleolus, it connects with the Spleen Meridian.

A branch runs along the lateral aspect of the tibia upward to the top of the head and converges with the other Yang Meridians on the head and neck.

From there it runs downward to connect with the throat.



Stomach

29

Stomach (ST)

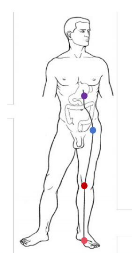
ST 40 Fenglong

The Collateral of Outer Circuit(4)

- **The Collateral of the Spleen Meridian of Foot-Taiyin**

It branches out at Gongsun (SP 4), one cun posterior to the base of the first metatarsal bone and then joins the Stomach Meridian.

A branch runs upward to the abdomen and connects with the stomach and intestines.



Spleen

30

Spleen (SP)

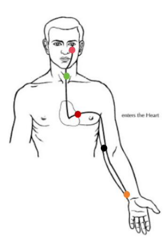
SP 4 Gongsun

The Collateral of inner Circuit(1)

- **The Collateral of the Heart Meridian of Hand-Shaoyin**

It branches out at Tongli (HT 5). One cun above the transverse crease of the wrist, it connects with the Small Intestine Meridian of Hand Taiyang.

About one and a half cun above the wrist, it again follows the meridian and enters the heart; it then runs to the root of the tongue and connects with the eye.



Heart

39

Heart (HT)

HT 5 Tongli

The Collateral of inner Circuit(2)

- **The Collateral of the Small Intestine Meridian of Hand--Taiyang**

It originates from Zhizheng (SI 7). Five cun above the wrist, it connects with the Heart Meridian.

Another branch runs upward, crosses the elbow and connects with Jianyu (LI 15).

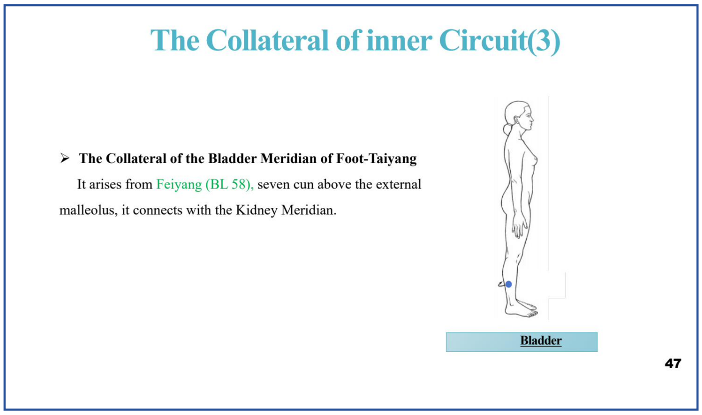
Small Intestine

40

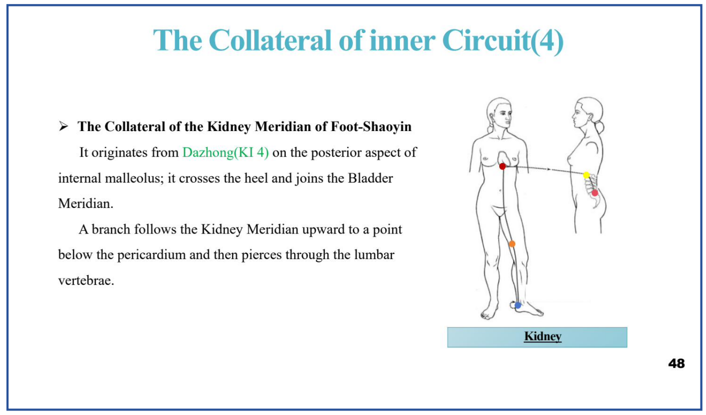
Small Intestine (SI)

SI 7 Zhizheng

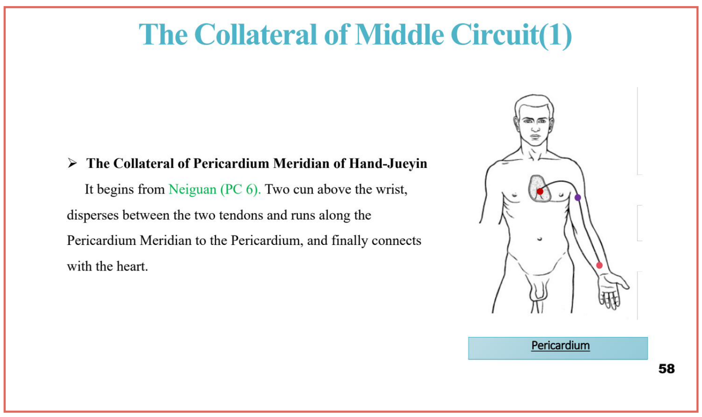
Collateral Pathway Diagrams -- Inner & Middle Circuit



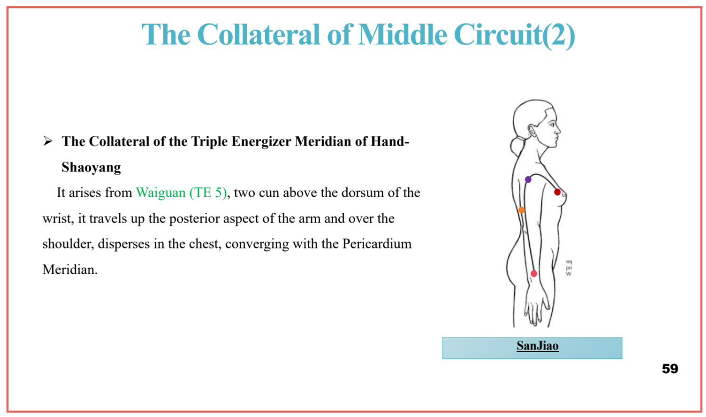
Bladder (BL)
BL 58 Feiyang



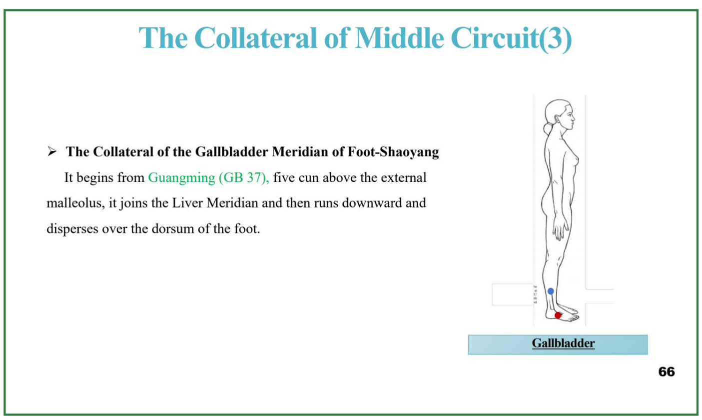
Kidney (KI)
KI 4 Dazhong



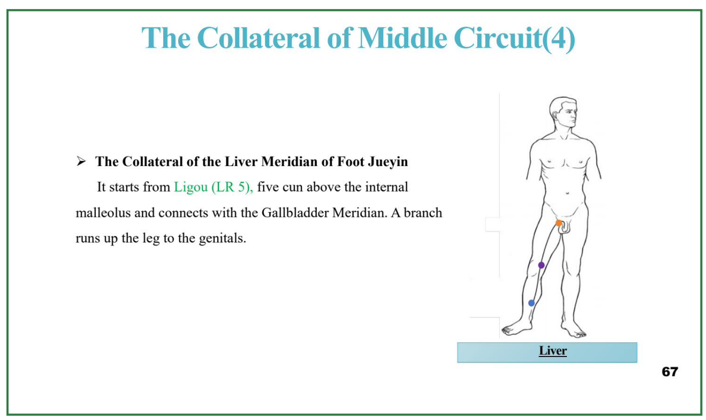
Pericardium (PC)
PC 6 Neiguan



Sanjiao (SJ)
SJ 5 Waiguan



Gallbladder (GB)
GB 37 Guangming



Liver (LR)
LR 5 Ligou

The 3 “Extra” Collaterals -- Completing the Set of 15

Collateral of the Conception Vessel (Ren Mai)

CV 15 Jiuwei

Course: Separates from the Governor Vessel at the lower end of the sternum; from Jiuwei (CV 15) it spreads over the abdomen.

Why it exists: Covers the FRONT midline -- balances the GV collateral on the back midline.

Collateral of the Governor Vessel (Du Mai)

GV 1 Changqiang

Course: Arises from Changqiang (GV 1) in the perineum; runs upward along both sides of the spine to the nape and spreads over the top of the head. At the scapular region it connects with the Bladder meridian and pierces through the spine.

Why it exists: Covers the BACK midline -- balances the CV collateral on the front midline.

Great (Major) Collateral of the Spleen

SP 21 Dabao

Course: Begins from Dabao (SP 21), emerges 3 cun below Yuanye (GB 22), and spreads through the chest and hypochondriac (lateral costal) region, gathering the blood all over the body.

Why it exists: Covers the LATERAL side of the chest -- the one area the 12 paired-meridian collaterals leave uncovered. This is WHY there are 15 collaterals, not 12: the body needs front (CV), back (GV), AND lateral (Spleen Great Luo) coverage to stay in harmony.

Picture It

CV's collateral covers the FRONT midline of the trunk. GV's collateral covers the BACK midline of the trunk. The Spleen's Great Collateral covers the LATERAL side of the chest -- the one region the other 14 collaterals leave unguarded. Front + Back + Side = complete surface coverage, which is the whole point of a system meant to “keep the whole body harmony” (Dr. Zhang, live).

FRONT	BACK	SIDE
CV 15 Jiuwei	GV 1 Changqiang	SP 21 Dabao
Ren Mai collateral	*Du Mai collateral*	*Spleen Great Collateral*

Memory Aid

The 12 paired-meridian Luo points are ALL located on the LIMBS (wrist or ankle region, per the “rule of thumb” above). The 3 extra Luo points are the exception -- all three are located on the TRUNK (sternum/Jiuwei, perineum/Changqiang, lateral chest/Dabao), because their job is to cover trunk surface area no limb-based point could reach.

The 12 Divergent Channels (Jing Bie)

The 12 Divergent Meridians (Jing Bie) originate from the 12 Primary Meridians, distribute throughout the chest, abdomen, and head, connect the superficial (exterior) and interior meridians, and strengthen the relationship with the Zang-Fu organs. Unlike the primary meridians, they have NO acupuncture points of their own and do NOT pertain to an organ -- their job is purely to reinforce/enhance the existing interior-exterior meridian relationships by running a deeper course.

Key Features

• Govern the INSIDE of the body (vs. collaterals, which control the body surface).
• Branch out from the 12 primary meridians; distributed mainly on the chest, abdomen, and head.
• Have NO acupuncture points of their own (points live in the skin; divergent channels run deep).
• Have no pertaining zang-fu organ of their own -- they strengthen the primary meridian's existing organ relationship instead.
• Function: enhance/strengthen the relationship between meridian and zang-fu organs, and supplement pathway coverage the primary meridian doesn't reach.

The Li - He - Chu - Merge Framework

Every Divergent Meridian follows the same 4-part running structure -- Chinese: Li – He – Chu – He (again). English glosses used in lecture: LI = beginning (where it branches off the primary meridian), HE = organs/systems the deep course involves, CHU = exiting (where it re-emerges to the surface), and finally it MERGES back into its paired YANG primary meridian. All 6 Yin divergent meridians merge into their paired Yang meridian at the head/face -- the Yin divergent channels never resurface as separate Yin channels.

Master Table -- All 12 at a Glance

MERIDIAN	BEGINNING	EXITING	MERGES INTO
Lung (LU)	The axilla	Supraclavicular fossa	Hand-Yangming (Large Intestine)
Large Intestine (LI)	Hand / Spine	Supraclavicular fossa	Hand-Yangming (itself -- LI is the Yang partner)
Stomach (ST)	Thigh, abdomen	Mouth	Foot-Yangming (itself -- ST is the Yang partner)
Spleen (SP)	Thigh	Throat	Foot-Yangming (Stomach)
Heart (HT)	Axillary fossa	Face	Hand-Taiyang (Small Intestine)
Small Intestine (SI)	Shoulder joint	Axilla / abdomen	Hand-Taiyang (itself -- SI is the Yang partner)
Bladder (BL)	Popliteal fossa	Neck	Foot-Taiyang (itself -- BL is the Yang partner)
Kidney (KI)	Popliteal fossa	Nape	Foot-Taiyang (Bladder)
Pericardium (PC)	Chest, 3 cun below the axilla	Behind the ear	Hand-Shaoyang (Sanjiao)
Sanjiao (SJ)	Vertex	Supraclavicular fossa	Hand-Shaoyang (itself -- SJ is the Yang partner)
Gallbladder (GB) *	Lateral thigh, hip joint, abdomen/pelvic r...	Face / outer canthus	Foot-Shaoyang (itself -- GB is the Yang partner)
Liver (LR) *	Instep of the foot	Pubic region	Foot-Shaoyang (Gallbladder)

** GB/LR divergent channels: self-study slide content, not reached live.*

Why the Divergent Channels Matter Clinically

Because a Divergent Channel runs DEEPER than its primary meridian and reaches organs/systems the primary meridian's own course doesn't directly pass through, it explains symptom patterns that otherwise look like they “shouldn't” belong to a given channel -- e.g. why Stomach-channel points can address disorders of the eye (ST divergent channel reaches the eye), or why Small-Intestine points can be chosen for heart-adjacent chest symptoms (SI divergent channel reaches the heart).

6 Confluence Pairs at a Glance

LU + LI · ST + SP · HT + SI · BL + KI · PC + SJ · GB + LR

LU + LI Divergent Confluence

Lung (LU)

Beginning:
The axilla (from the Lung meridian)

Organs/Systems:
Lung, Large Intestine, Throat

Exiting:
Supraclavicular fossa

Merges Into:
Hand-Yangming (Large Intestine)

Course, Step by Step

1. Deriving from the Lung meridian in the axilla -> chest, anterior to the Pericardium meridian -> the lung -> the large intestine
2. A branch: emerges at the clavicle, runs upward from the lung -> the throat -> merges into the Large Intestine meridian

Large Intestine (LI)

Beginning:
Hand / Spine

Organs/Systems:
Spine, Large Intestine, Lung, Throat

Exiting:
Supraclavicular fossa

Merges Into:
Hand-Yangming (itself -- LI is the Yang partner)

Course, Step by Step

1. Deriving from the LI meridian on the hand -> shoulder -> arm -> breast
2. Branch: shoulder -> top of the shoulder -> nape -> the spine, then downward to the large intestine and the lung
3. Branch: shoulder -> emerging at the supraclavicular fossa -> the throat -> the LI meridian

Clinical Relevance

Clinical relevance: because the LU divergent channel passes anterior to the Pericardium meridian on its way to the lung, and the LI divergent channel reaches all the way back to the spine, this pair explains why LU/LI points can reach both chest (PC-adjacent) and upper-back complaints, not just their own organ's territory.

Compare the Pair

• Both channels exit to the surface at the same landmark: Supraclavicular fossa.

• Both ultimately merge into the Large Intestine channel -- every Yin divergent channel merges into its paired Yang channel, never resurfacing as a separate Yin pathway.

ST + SP Divergent Confluence

Stomach (ST)

Beginning:
Thigh, abdomen (from the Stomach meridian)

Organs/Systems:
Abdomen, stomach, spleen, heart, esophagus, mouth, nose, eye

Exiting:
Mouth

Merges Into:
Foot-Yangming (itself -- ST is the Yang partner)

Course, Step by Step

1. From the ST meridian (thigh) -> abdomen -> stomach -> spleen -> through the heart -> alongside the esophagus -> the mouth -> upward beside the nose -> the eye -> merges into the ST meridian

Spleen (SP)

Beginning:
Thigh (from the Spleen meridian)

Organs/Systems:
Throat, tongue

Exiting:
Throat (converges upward with the ST divergent channel)

Merges Into:
Foot-Yangming (Stomach)

Course, Step by Step

1. Deriving from the SP meridian on the thigh -> converges upward with the Divergent Meridian of the Stomach -> the throat -> the tongue

Clinical Relevance

Clinical relevance: the ST divergent channel is the only one of the 12 that runs THROUGH the heart on its way to the mouth/eye -- this is the anatomical basis for using ST points in some heart-adjacent chest patterns, in addition to their expected digestive use.

Compare the Pair

- Stomach exits at Mouth; Spleen exits at Throat -- different surface landmarks.
- Both ultimately merge into the Stomach channel -- every Yin divergent channel merges into its paired Yang channel, never resurfacing as a separate Yin pathway.

HT + SI Divergent Confluence

Heart (HT)

Beginning:
Axillary fossa (from the Heart meridian)

Organs/Systems:
Heart, chest, throat

Exiting:
Face

Merges Into:
Hand-Taiyang (Small Intestine)

Course, Step by Step

1. From the HT meridian (axillary fossa) -> the chest -> the heart -> the throat -> emerging on the face -> the SI meridian (inner canthus)

Small Intestine (SI)

Beginning:
Shoulder joint (from the Small Intestine meridian)

Organs/Systems:
Heart, small intestine

Exiting:
Axilla / abdomen

Merges Into:
Hand-Taiyang (itself -- SI is the Yang partner)

Course, Step by Step

1. From the SI meridian (shoulder joint) -> the axilla -> the heart -> the abdomen -> the SI meridian
2. Specific feature: the SI divergent/primary meridian reaches BOTH the inner and outer canthus of the eye -- unique among the meridians

Clinical Relevance

Clinical relevance: both divergent channels begin at the SAME landmark (axillary fossa / shoulder joint area) and both pass through the heart -- reinforcing why HT and SI points are so often combined for chest/cardiac-adjacent and mouth-area symptom patterns.

Compare the Pair

• Heart exits at Face; Small Intestine exits at Axilla / abdomen -- different surface landmarks.

• Both ultimately merge into the Small Intestine channel -- every Yin divergent channel merges into its paired Yang channel, never resurfacing as a separate Yin pathway.

BL + KI Divergent Confluence

Bladder (BL)

Beginning:
Popliteal fossa (from the Bladder meridian)

Organs/Systems:
Bladder, kidneys, spine, cardiac region

Exiting:
Neck

Merges Into:
Foot-Taiyang (itself -- BL is the Yang partner)

Course, Step by Step

1. Deriving from the BL meridian in the popliteal fossa -> 5 cun below the sacrum -> the anal region -> the bladder -> the kidneys -> the spine -> the cardiac region -> emerging at the neck -> the BL meridian

Kidney (KI)

Beginning:
Popliteal fossa (from the Kidney meridian)

Organs/Systems:
Kidney, root of the tongue

Exiting:
Nape (merges upward with the BL divergent channel)

Merges Into:
Foot-Taiyang (Bladder)

Course, Step by Step

1. From the KI meridian (popliteal fossa) -> upward, intersecting the Divergent Meridian of the Bladder (thigh) -> the kidney -> crossing the Dai (Belt) Meridian at the level of T7 -> the root of the tongue -> emerging at the nape -> the BL meridian

Clinical Relevance

Clinical relevance: the KI divergent channel is the only one that crosses the Dai (Belt) Meridian along its course (at the level of T7) -- a rare direct link between a primary meridian's divergent channel and an Extraordinary Vessel.

Compare the Pair

• Bladder exits at Neck; Kidney exits at Nape -- different surface landmarks.

• Both ultimately merge into the Bladder channel -- every Yin divergent channel merges into its paired Yang channel, never resurfacing as a separate Yin pathway.

PC + SJ Divergent Confluence

Pericardium (PC)

Beginning:
Chest, 3 cun below the axilla (from the Pericardium meridian)

Organs/Systems:
Sanjiao (Triple Energizer)

Exiting:
Behind the ear (via a branch)

Merges Into:
Hand-Shaoyang (Sanjiao)

Course, Step by Step

1. From the PC meridian (3 cun below the axilla) -> the chest -> the Sanjiao
2. Branch: ascending across the throat -> emerging behind the ear -> the Sanjiao meridian

Sanjiao (SJ)

Beginning:
Vertex (from the Sanjiao meridian)

Organs/Systems:
Upper, middle, and lower Jiao; chest

Exiting:
Supraclavicular fossa

Merges Into:
Hand-Shaoyang (itself -- SJ is the Yang partner)

Course, Step by Step

1. From the SJ meridian (vertex) -> the chest -> the upper, middle, and lower Jiao -> emerging at the supraclavicular fossa -> the SJ meridian

Clinical Relevance

Clinical relevance: both divergent channels are unusually short (chest-to-Sanjiao and vertex-to-chest) compared to the limb-spanning divergent channels of most other pairs -- consistent with PC/SJ's role as the Ministerial Fire pair governing the Triple Jiao itself.

Compare the Pair

• Pericardium exits at Behind the ear; Sanjiao exits at Supraclavicular fossa -- different surface landmarks.

• Both ultimately merge into the Sanjiao channel -- every Yin divergent channel merges into its paired Yang channel, never resurfacing as a separate Yin pathway.

GB + LR Divergent Confluence

Self-study slide content (not reached live)

Gallbladder (GB)

Beginning:
Lateral thigh, hip joint, abdomen/pelvic region (from the Gallbladder meridian)

Organs/Systems:
Gallbladder, liver, heart, esophagus, eye

Exiting:
Face / outer canthus

Merges Into:
Foot-Shaoyang (itself -- GB is the Yang partner)

Course, Step by Step

1. From the GB meridian (thigh) -> the hip joint -> the lower abdomen -> the pelvic region -> converging with the Divergent Meridian of the Liver -> between the lower ribs -> the gallbladder -> the liver -> upward through the heart -> esophagus -> the face -> the eye -> the GB meridian (outer canthus)

Liver (LR)

Beginning:
Instep of the foot (from the Liver meridian)

Organs/Systems:
Pubic region (converges with the GB divergent channel)

Exiting:
Pubic region

Merges Into:
Foot-Shaoyang (Gallbladder)

Course, Step by Step

1. Deriving from the LR meridian on the instep -> the pubic region -> converging with the Divergent Meridian of the Gallbladder

Clinical Relevance

Clinical relevance: the GB divergent channel is the only one that explicitly passes through BOTH paired organs (gallbladder AND liver) on a single course -- self-study content; confirm this reading with Dr. Zhang's review before treating as final.

Compare the Pair

• Gallbladder exits at Face / outer canthus; Liver exits at Pubic region -- different surface landmarks.

• Both ultimately merge into the Gallbladder channel -- every Yin divergent channel merges into its paired Yang channel, never resurfacing as a separate Yin pathway.

The 12 Muscle (Sinew) Regions -- Definition & Rules

The 12 Muscle (Sinew) Regions, Jing Jin, are a newer nomenclature layered on top of the primary meridians -- their pathways mainly follow the course of the corresponding primary meridian, just over a WIDER area (a region, not a line). They originate at the extremities of the limbs and ascend to the head and trunk. They do NOT go inside the body and do NOT connect with the zang-fu organs -- they connect only with muscles, tendons, and bones/joints.

Functions

- Nourish the muscles; clinical significance is mainly muscular -- Bi (painful obstruction) syndrome, contracture, stiffness, spasm, and muscular atrophy.
- Connect all the bones and joints of the body.
- Maintain the normal range of motion.
- Run from the extremities of the limbs toward the head and trunk (never the reverse).

The 4 Structural Pattern Rules

- All 3 Yang Muscle Regions of the FOOT (anterior/lateral/posterior aspects of the trunk) connect with the EYES.
- All 3 Yin Muscle Regions of the FOOT connect with the GENITAL region.
- All 3 Yang Muscle Regions of the HAND connect with the ANGLE OF THE FOREHEAD.
- All 3 Yin Muscle Regions of the HAND connect with the THORACIC CAVITY (chest).

Clinical Note (from live Q&A)

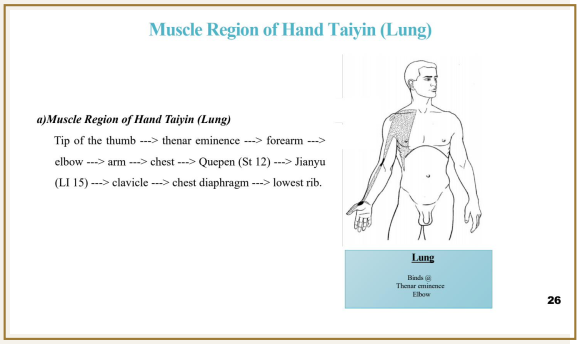
Clinical use (from Q&A in lecture): because a single acupoint is small, but a Muscle Region covers a whole area along the channel's course, practitioners can treat local muscle/joint pain and motor dysfunction (including conditions like osteoarthritis-related pain, per the massage-therapist student's question) by stimulating anywhere along the Muscle Region, not just the exact acupoint -- this is part of the theoretical basis for techniques like dry needling.

Muscle Regions -- Outer Circuit (LU/LI, ST/SP)

Hand-Taiyin (Lung)

Binds at: Thenar eminence, elbow

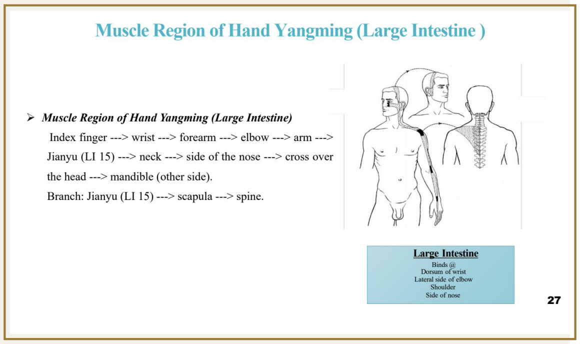
Pathway: Tip of thumb -> thenar eminence -> forearm -> elbow -> arm -> chest -> Quepen (ST 12) -> Jianyu (LI 15) -> clavicle -> chest diaphragm -> lowest rib.



Hand-Yangming (Large Intestine)

Binds at: Dorsum of wrist, lateral elbow, shoulder, side of nose

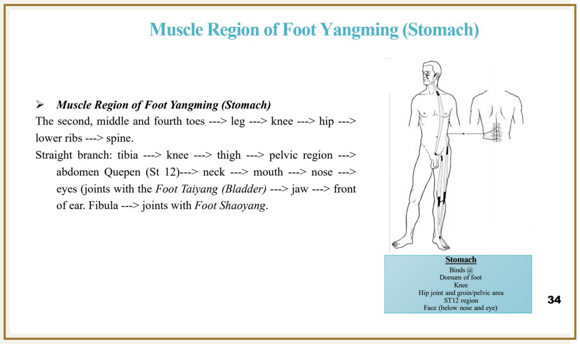
Pathway: Index finger -> wrist -> forearm -> elbow -> arm -> Jianyu (LI 15) -> neck -> side of nose -> crosses over the head -> mandible (other side). Branch: Jianyu -> scapula -> spine.



Foot-Yangming (Stomach)

Binds at: Knee, hip, Quepen ST12, eyes

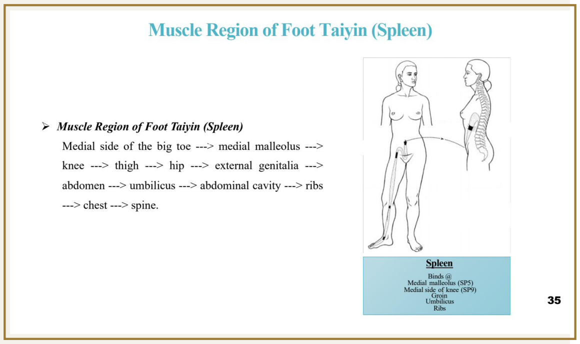
Pathway: 2nd/middle/4th toes -> leg -> knee -> hip -> lower ribs -> spine. Straight branch: tibia -> knee -> thigh -> pelvic region -> abdomen -> Quepen (ST 12) -> neck -> mouth -> nose -> eyes (joins Foot-Taiyang/BL) -> jaw -> front of ear. Fibula branch -> joins the lateral system.



Foot-Taiyin (Spleen)

Binds at: Medial malleolus (SP 5), medial knee (SP 9), groin, umbilicus, ribs

Pathway: Medial side of big toe -> medial malleolus -> knee -> thigh -> hip -> external genitalia -> abdomen -> umbilicus -> abdominal cavity -> ribs -> chest -> spine.

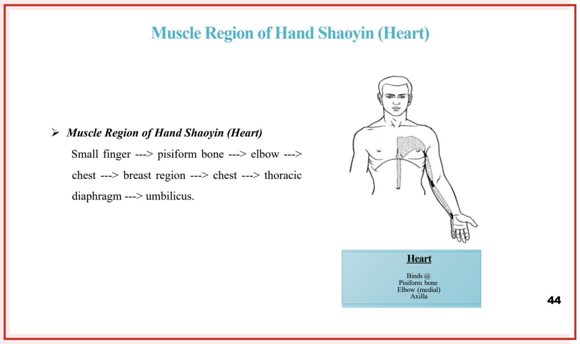


Muscle Regions -- Inner Circuit (HT/SI, BL/KI)

Hand-Shaoyin (Heart)

Binds at: Pisiform bone, medial elbow, axilla

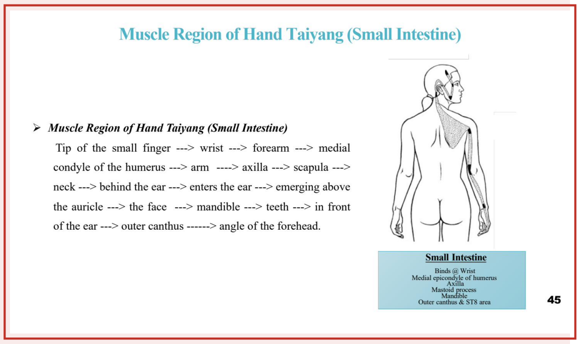
Pathway: Small finger -> pisiform bone -> elbow -> chest -> breast region -> chest -> thoracic diaphragm -> umbilicus.



Hand-Taiyang (Small Intestine)

Binds at: Wrist, medial epicondyle, axilla, mastoid process, mandible, outer canthus/ST8 area

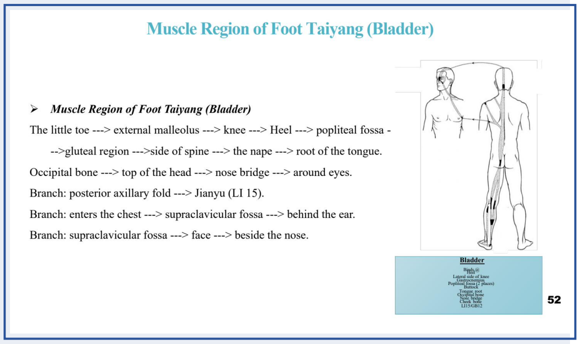
Pathway: Tip of small finger -> wrist -> forearm -> medial condyle of humerus -> arm -> axilla -> scapula -> neck -> behind the ear -> enters the ear -> emerges above the auricle -> the face -> mandible/outer canthus (ST 8 area).



Foot-Taiyang (Bladder)

Binds at: Popliteal fossa, nape, occiput, eyes

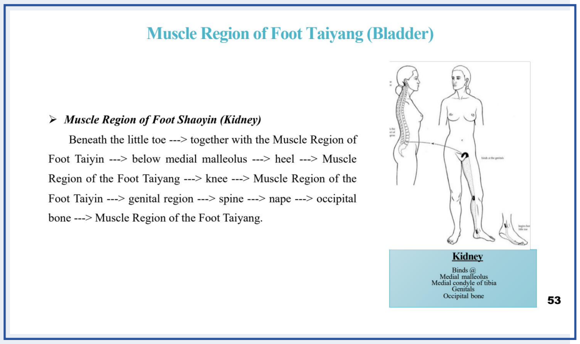
Pathway: Little toe -> external malleolus -> knee -> heel -> popliteal fossa -> gluteal region -> side of spine -> the nape -> root of the tongue -> occipital bone -> top of head -> nose bridge -> around the eyes. Branch: posterior axillary fold -> Jianyu (LI 15). Branch: enters the chest.



Foot-Shaoyin (Kidney)

Binds at: Below medial malleolus, knee, genital region, occiput

Pathway: Beneath the little toe -> (together with Foot-Taiyin) below the medial malleolus -> heel -> (with Foot-Taiyang) knee -> (with Foot-Taiyin) genital region -> spine -> nape -> occipital bone -> (with Foot-Taiyang).



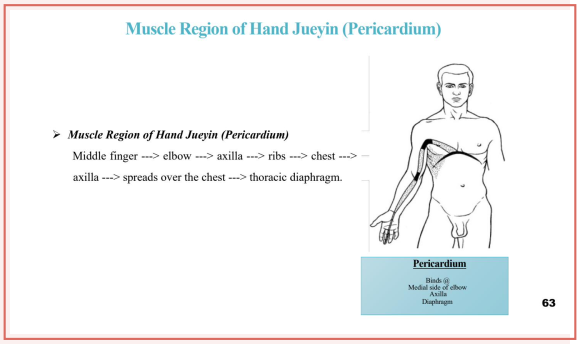
Note: the source lecture slide for the Kidney diagram above is mistitled "Muscle Region of Foot Taiyang (Bladder)" in Dr. Zhang's deck, but its body text and image content are the Kidney muscle region -- a source-slide labeling error, flagged here rather than silently corrected in the image itself. The text card is labeled correctly (Kidney).

Muscle Regions -- Middle Circuit (PC/SJ, GB/LR)

Hand-Jueyin (Pericardium)

Binds at: Medial elbow, axilla, diaphragm

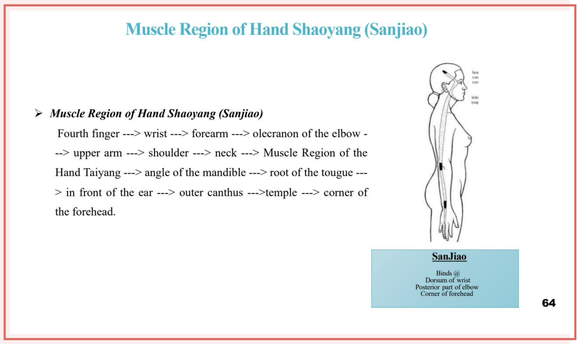
Pathway: Middle finger -> elbow -> axilla -> ribs -> chest -> axilla -> spreads over the chest -> thoracic diaphragm.



Hand-Shaoyang (Sanjiao)

Binds at: Dorsum of wrist, posterior elbow, corner of forehead

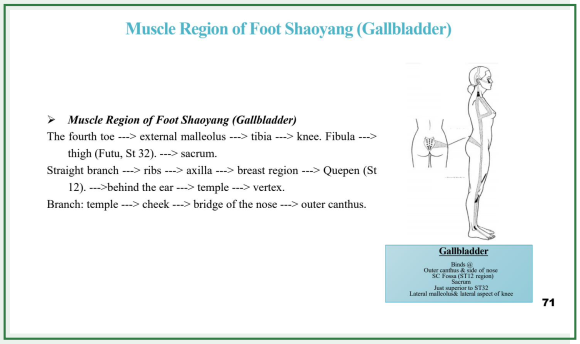
Pathway: 4th finger -> wrist -> forearm -> olecranon -> upper arm -> shoulder -> neck -> (joins Hand-Taiyang) -> angle of mandible -> root of the tongue -> in front of the ear -> outer canthus -> temple -> corner of the forehead.



Foot-Shaoyang (Gallbladder) [self-study]

Binds at: Outer canthus/side of nose, SC fossa (ST12 region), sacrum, above ST32, lateral malleolus/lateral knee

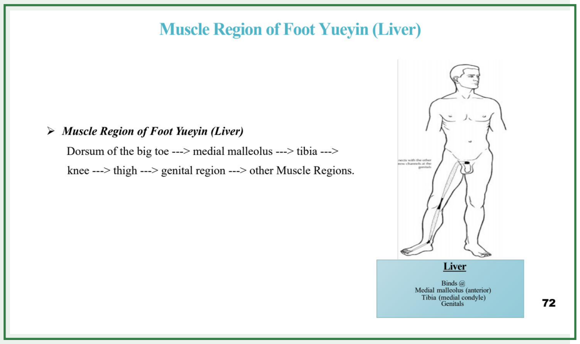
Pathway: 4th toe -> external malleolus -> tibia -> knee. Fibula -> thigh (Futu, ST 32) -> sacrum. Straight branch -> ribs -> axilla -> breast region -> Quepen (ST 12) -> behind the ear -> temple -> vertex. Branch: temple -> cheek -> outer canthus.



Foot-Jueyin (Liver) [self-study]

Binds at: Anterior medial malleolus, medial condyle of tibia, genitals

Pathway: Dorsum of big toe -> medial malleolus -> tibia -> knee -> thigh -> genital region -> joins other muscle regions.



The 12 Cutaneous Regions (Pi Bu)

Self-study slide content (slides 79-83) -- not reached live this lecture.

The 12 Cutaneous Regions (Pi Bu) are the parts of the 12 meridians reflected on the body surface, where the qi of the meridians is distributed -- the outermost layer of the human body. Su Wen (Chapter 56): “The Cutaneous Regions are the part of the meridian system located in the superficial layers of the body, marked by the regular meridians.”

Three Functions (Su Wen, Chapter 56)

Protects against exogenous pathogen invasion

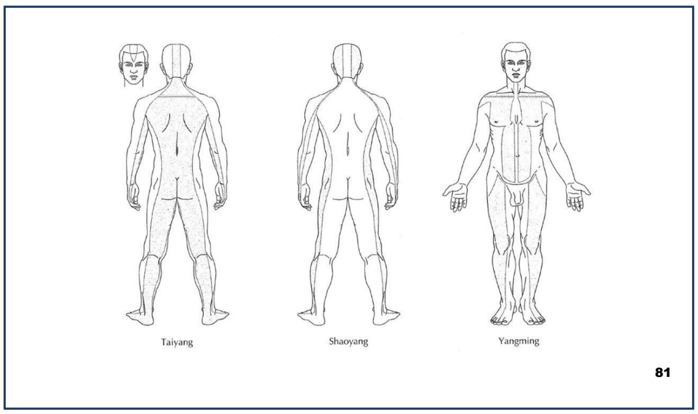
Su Wen Ch.56: “Skin is the place where the meridians are distributed. When the pathogen attacks the skin, the sweat pores open, and the pathogen may advance toward the collaterals, meridians, and zang-fu organs through the sweat pore.” Transmission order: Skin -> Collaterals -> Meridians -> Fu organs -> Zang organs.

Projects symptoms/signs of internal disease onto the surface

Su Wen Ch.56: blue-colored skin signals local pain; dark-colored skin indicates blocked qi and blood; yellow-to-red skin signals heat syndromes; white skin signals cold syndromes.

Diagnostically and therapeutically interactive

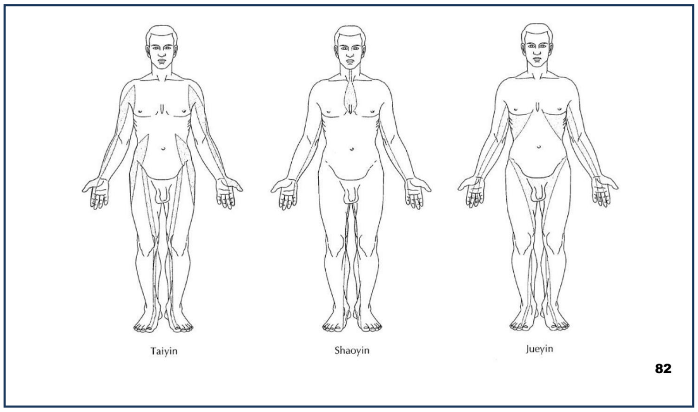
Because the cutaneous region is the most superficial layer, it is both the first line read for diagnosis (skin color/texture change) and the first line treated (very superficial needling, e.g. intradermal/press needles, work through this layer).



Lecture Fig. — 3 Yang Cutaneous Regions

Lecture8vivian1119.pdf, slide 82 (Dr. Zhang)

Cutaneous Regions (cont.) + Week 7 Recap + Exam Focus



Lecture Fig. — 3 Yin Cutaneous Regions

Lecture8vivian1119.pdf, slide 83 (Dr. Zhang)

The 6 Divisions

3 Yang (posterior/lateral view)

- Taiyang (BL/SI territory)
- Shaoyang (GB/SJ territory)
- Yangming (ST/LI territory)

3 Yin (anterior view)

- Taiyin (LU/SP territory)
- Shaoyin (HT/KI territory)
- Jueyin (PC/LR territory)

Transmission order (disease): Skin -> Collaterals -> Meridians -> Fu organs -> Zang organs -- deeper each step, matching how superficial-to-deep these four new systems (Cutaneous -> Collaterals -> Divergent -> organs) are taught this week.

Week 7 Recap — the 8 Confluent Points (bridge review)

Dr. Zhang opened Lecture 8 with a live Q&A review of the 8 Extraordinary Vessels' confluent points before starting new content -- see the Week 8 Cram Sheet / PLA for the full Q&A. Reference table (Dr. Zhang's own review slide):

Confluent points

Point	Primary Meridian	Extraordinary Vessel	Classic Use
Houxi (SI3)	Small Intestine	Governor Vessel (*Du Mai*)	Spine pain, anxiety, fever
Gongsun (SP4)	Spleen	Penetrating Vessel (*Chong Mai*)	Digestive issues, period cramps
Shenmai (BL62)	Bladder	Yang Heel Vessel (*Yang Qiao Mai*)	Insomnia, back stiffness
Zhaohai (KD6)	Kidney	Yin Heel Vessel (*Yin Qiao Mai*)	Dry throat, hormonal balance
Lieque (LU7)	Lung	Conception Vessel (*Ren Mai*)	Cough, grief, skin issues
Neiguan (PC6)	Pericardium	Yin Linking Vessel (*Yin Wei Mai*)	Heart palpitations, stress
Waiguan (SJ5)	Triple Energizer	Yang Linking Vessel (*Yang Wei Mai*)	Migraines, immune support
Zulinqi (GB41)	Gallbladder	Girdle Vessel (*Dai Mai*)	Hip pain, gallbladder disorders

Lecture8vivian1119.pdf, slide 78 (Dr. Zhang) — Week 7 confluent points review

Dr. Zhang's Own Exam-Focus Summary (verbatim, slide 84)

Dr. Zhang's own "Next Quiz" summary slide (verbatim, slide 84): "The exam focuses on the pathways, connections, and special features of the meridians. You should know which organs each meridian is associated with, their starting and ending points, and key landmarks they pass -- such as the ear, external genitalia, pubic region, and waist. Pay attention to the unique traits of certain meridians like the Dai Vessel encircling the waist, the Governor Vessel starting from the perineum, and the Gallbladder Meridian's zig-zag path along the head. Also review paired relationships between meridians, the composition of the fifteen collaterals, and how channels connect with each other." Keyword list from the same slide: KI, Pericardium, LR, GB, TE (SJ), Belt Vessel, GV, 15 Collaterals, GB.